

S A K H I H E A L T H

Growth Playbook: Path to ₹8.5Cr (\$1M) ARR

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The Paradox: India's women are digitally active and wellness-aware — yet **only 1.9% have ever been screened for cervical cancer** (NFHS-5). Sakhi bridges this gap by starting with daily wellness habits and progressively deepening into preventive screening and care pathways.

650M+

Women in India

<2%

Ever screened
for cervical cancer

\$3.87B

India FemTech
by 2030 (18% CAGR)

17.05%

India femtech
market CAGR

PERSONA 1: URBAN EARLY ADOPTER

"Priya" — 30, IT Professional, Bangalore

Job-to-be-Done: Stay healthy proactively without adding complexity to an already packed schedule

Trigger: Friend diagnosed with PCOS at 28; HR email about women's health week

Barriers: Time scarcity (60-hr weeks), fear of bad news, no single trusted platform

Anxieties: "Will they upsell me?", "Is my data safe?", embarrassment around intimate health topics

Trust Drivers: Doctor-backed content, peer reviews, privacy-first design, no-spam promise

Context: Smartphone 6+ hrs/day, uses HealthifyMe for diet, WhatsApp-native, pays for Spotify/Netflix

KEY INSIGHT

Priya will try a wellness app on impulse — but screening requires a trust bridge. She needs 3-4 weeks of value before she'll share health data or book a test.

BEHAVIORAL PROFILE

Willingness to pay:

₹199-399/mo for wellness; ₹1,500-2,500 for screening if trusted

Digital comfort:

High — already pays for 2-3 subscriptions

Referral likelihood:

Moderate — shares health tips in WhatsApp groups

"Meena" — 38, Homemaker + Small Business, Surat

Job-to-be-Done: Protect family health while managing household; prevent illness that disrupts family

Trigger: Relative diagnosed with breast cancer; community ASHA worker mentions screening camp

Barriers: Husband/in-law permission, cost sensitivity, lack of nearby diagnostics, low health literacy

Anxieties: "Doctor will find something bad", "Family will worry", "I can't afford treatment"

Trust Drivers: Community endorsement, Gujarati/Hindi content, ASHA/doctor referral, family-inclusive design

Context: Smartphone 3-4 hrs/day, WhatsApp primary app, shops on Meesho, consumes YouTube health videos

KEY INSIGHT

Meena won't download an app on her own. She needs a trusted intermediary (ASHA worker, community leader, or corporate camp) to introduce Sakhi. Once in, community features drive retention.

URBAN vs SEMI-URBAN: WHY WE START URBAN

	Urban (Priya)	Semi-Urban (Meena)
Smartphone	~68%	~38%
CAC	₹80-150	₹200-400
WTP (screening)	₹2,000-3,000	₹800-1,500
Trust pathway	Content → App	Community → App
Year 1 focus?	YES (beachhead)	Year 2+ (partner-led)

Metric Stack: From Impression to Revenue

ACQUISITION	Key Metric: Install rate (target: 25%) Leading: Content CTR on Instagram/YT	Leakage: High bounce from generic health ads — women don't trust new brands
ACTIVATION	Key Metric: Day 7 retained (target: 45%) Leading: Risk profile completed (Day 0-3)	Leakage: Onboarding asks too much data before delivering value
RETENTION	Key Metric: WAU/MAU (target: 35%) Leading: 3+ sessions/week by Day 14	Leakage: Wellness-only users plateau — no deepening trigger
MONETIZATION	Key Metric: Screening conversion (target: 5%) Leading: Risk score viewed + reminder set	Leakage: Price shock at screening stage; no local lab partner
REFERRAL	Key Metric: Viral coeff. (target: 0.3) Leading: Shared health tip or challenge invite	Leakage: Stigma — women don't share 'health app' publicly

What We Optimize — and What We Don't

PRIMARY: Weekly Active Users (WAU) who complete ≥ 1 health action

Why WAU over MAU: Health is a weekly habit, not monthly. WAU who take a health action (log, track, read, assess) are 4x more likely to convert to screening within 90 days. This metric is upstream of revenue but directly causal.

SECONDARY METRICS

1. Screening bookings/mo
→ Revenue proxy
2. Risk profiles completed
→ Conversion predictor
3. Day 30 retention %
→ Cohort health

NOT OPTIMIZED IN YEAR 1: ARPU

Maximizing ARPU in Year 1 would mean aggressive upselling, premium paywalls, and high screening prices — all of which destroy trust in a women's health platform. We set floor ARPU (₹150/mo blended) and optimize for WAU volume and screening conversion rate instead. ARPU optimization begins Month 18+ when trust is established.

METRIC LADDER TO \$1M

WAU → Risk Profile Completion → Screening Reminder Set → Screening Booked → Revenue

~45K MAU × 35% WAU ratio × 5% annual screening conversion × ₹2,000 avg ticket + B2B revenue + subscriptions = ₹8.5Cr (\$1M)

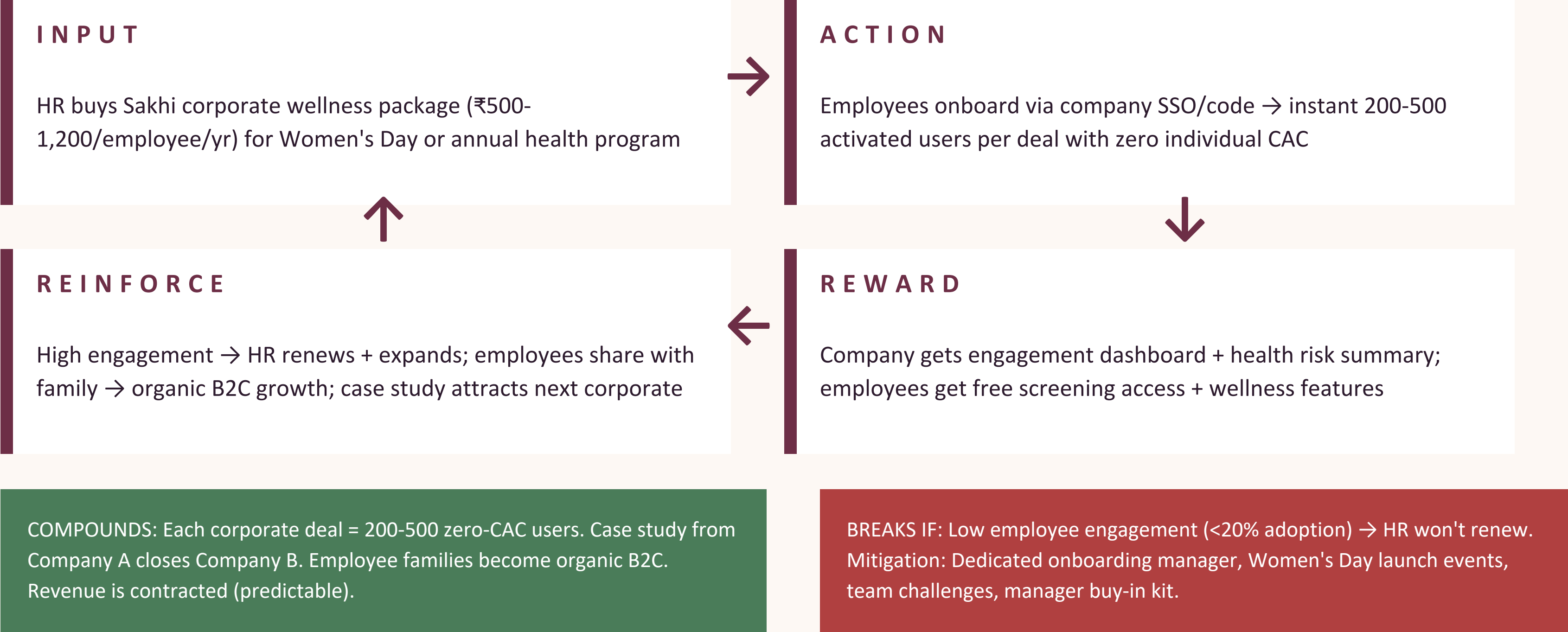
GROWTH LOOP 1: ENGAGEMENT LOOP

Wellness Habit → Health Identity → Screening Readiness



GROWTH LOOP 2: DISTRIBUTION + MONETIZATION

B2B2C Corporate Wellness Flywheel



Channel Mix + Trade-offs (Limited Budget)

1. Content-Led SEO + Instagram Reels Health tips in Hindi/English, PCOS/thyroid myth-busting, period health. Target: 50K organic visitors/mo by M12. SEO on "women's health checkup near me" + "PCOS symptoms" Effective CAC: ₹30-60	ORGANIC — 40% of budget Trade-off: Slow to build (3-6 mo), but highest LTV users. Quality > speed.
2. B2B Corporate Wellness Outreach Direct outreach to HR heads at IT/BPO firms in Bangalore, Pune, Mumbai. Offer free pilot (50 employees × 30 days). Convert to paid at ₹800/employee/yr. Effective CAC: ₹0 per user (paid by employer)	UNFAIR WEDGE — 35% of budget Trade-off: Longer sales cycle (2-4 mo) but zero-CAC users + contracted revenue. B2B > B2C for unit economics.
3. Targeted Meta/Google Ads Instagram + Google Search targeting women 25-45 in Tier 1-2 cities. Creative: "2-min health risk check" lead magnet. Retarget content readers. CAC: ₹100-180	PAID — 25% of budget Trade-off: Fast but lower retention (Day 30: ~15% vs 25% organic). Speed > quality, used to fill top of funnel.

Aha Moment + Day 0 / 7 / 30 Design

AHA MOMENT: *"I just learned something specific about MY health that I didn't know — and it took only 2 minutes."*

DAY 0First Value in 90 Seconds	DAY 7Habit Anchored	DAY 30Trust Bridge Crossed
→ Quick 5-question health snapshot (no sign-up wall) → Instant personalized risk insight + 1 actionable tip → Light account creation post-value (email/phone only) → Set first health reminder (water/sleep/walk) Target: 55% complete risk snapshot	→ 3+ wellness logs completed (streak started) → Deeper health profile quiz unlocked (menstrual, diet, family history) → First community challenge invite ("7-Day Hydration") → Push: personalized insight based on Day 0-7 data Target: 45% Day 7 retained	→ Full risk profile complete → personalized screening plan → First screening reminder set (soft: "Would you like to...") → Health dashboard shows progress over 30 days → Referral prompt: "Share your health journey with a friend" Target: 22% Day 30 retained

Habit Mechanic: "Sakhi Score" — Health Identity System

THE SAKHI SCORE

A composite health engagement score (0-100) that reflects wellness habits, screening status, and health knowledge — NOT a medical score. Designed ethically to motivate without creating anxiety. Daily habits logged (40%) + Screening up-to-date (30%) + Knowledge quizzes (20%) + Community participation (10%)

Ethics guard:
Score never decreases for missing a day. No shame mechanics. Language:

RETENTION PREDICTORS

- 1. 3+ sessions in first week → 2.5x more likely to retain D30
- 2. Risk profile completed by Day 10 → 3x screening conversion
- 3. Joined a community challenge → 40% higher WAU
- 4. Set 2+ health reminders → 1.8x retention

NUDGE STRATEGY (Ethical Health Nudging)

Timing	Type	Example	CTR Target
Day 1-3	Value nudge	"Your personalized hydration tip is ready"	10-12%
Day 7-14	Streak nudge	"5-day streak! You're building a great habit"	8-10%
Day 14-21	Insight nudge	"Based on your data, here's your wellness trend"	7-9%
Day 21-30	Screening nudge	"Women with your profile benefit from X check"	5-7%
Day 30+	Re-engagement	"Your monthly health summary is ready"	6-8%

Primary + Secondary Levers + Ethics

PRIMARY: Screening Packages (B2C)

~45% of revenue by M36

Bundled screening packages (₹1,500-3,000): breast, cervical, NCD panels. Booked in-app, fulfilled by partner labs (Healthians, SRL, Thyrocare). Sakhi takes 20-25% commission.

Positive retention: screened users have 2x Day-90 retention (invested in outcomes). Risk: price shock if presented without progressive commitment.

SECONDARY: Corporate Wellness (B2B)

~35% of revenue by M36

Annual contracts at ₹500-1,200/employee/yr. Includes wellness access + 1 annual screening + engagement dashboard for HR. Target: 15-20 corporate clients by M24.

Contracted, predictable revenue. Anchor tenants reduce CAC to zero. Risk: long sales cycle (2-4 months).

TERTIARY: Premium Wellness Subscription

~15% of revenue by M36

Sakhi Pro at ₹199-299/mo: AI nutrition plans, expert consultations, advanced health dashboards, priority screening slots. Freemium → premium conversion target: 3-5%.

Recurring revenue improves LTV. Risk: competing with free YouTube/Instagram health content. Must deliver 10x value.

REJECTED (Ethics): (1) Selling user health data to pharma/insurance — violates trust covenant, would destroy the brand. (2) Diagnostic lab white-labeling with hidden margin — opaque pricing erodes trust. We commit to transparent pricing and data sovereignty.

3 Experiments Mapped to Funnel Leakage Points

EXP 1: Value-First Onboarding vs. Sign-Up-First

Targets: Activation: Install → First Action (target leakage: 40-60%)

H: Showing personalized health insight BEFORE requiring account creation will increase Day 0 activation by 20%+

Upside: If +20% activation → ~400 more activated users/month → compounds via engagement loop **Risk:** Variant may increase activation but collect less data upfront, reducing personalization quality. Mitigate: ask 3 essential Qs in quiz itself.

Design: A/B test: Control = sign-up first, then health quiz. Variant = health quiz first, then lightweight sign-up. 50/50 split. N=2,000 installs (1,000 per arm). Run 3 weeks.

EXP 2: Community Challenge vs. Solo Streak for D7 Retention

Targets: Retention: Day 1-7 drop-off (target leakage: 45% → improving to 55%)

H: Users invited to a 7-day group wellness challenge will have 25%+ higher Day 7 retention than solo streak users

Upside: If +25% D7 retention → dramatically improves lifetime value and screening conversion downstream **Risk:** Social features may deter privacy-conscious users. Mitigate: challenges are anonymous (no real names), opt-in only.

Design: A/B: Control = individual streak counter. Variant = 5-person mini-challenge ("Hydration Week"). N=1,500. Run 2 weeks. Measure Day 7 retention + sessions/week.

EXP 3: Progressive Screening Nudge vs. Direct Offer

Targets: Monetization: Engaged user → Screening booking (target: 3-7%)

H: A 3-step progressive nudge sequence (risk insight → reminder → booking) will convert 40%+ more users to screening than a direct discount offer

Upside: If +40% conversion → adds ~₹3L/mo revenue at scale. Validates progressive commitment thesis. **Risk:** Longer conversion cycle may reduce short-term revenue. Acceptable: trust > speed in health.

Design: A/B: Control = "Book your screening — 20% off" at Day 21. Variant = Day 14: risk insight, Day 21: reminder with social proof, Day 28: booking with personalized recommendation. N=1,000 eligible users. Run 6 weeks.

The Math: ₹8.5 Crore (\$1M) ARR by Month 30

Metric	M6	M12	M18	M24	M30	M36
Total Users (cumul.)	8K	22K	42K	70K	105K	150K
MAU	3K	10K	20K	35K	50K	65K
WAU (35% of MAU)	1K	3.5K	7K	12.3K	17.5K	22.8K
Screening bookings/mo	30	120	350	700	1,100	1,600
Screening rev/mo (₹)	₹60K	₹2.4L	₹7L	₹14L	₹22L	₹32L
B2B clients	2	6	12	18	22	28
B2B seats	400	1.5K	3.5K	6K	8.5K	11K
B2B rev/mo (₹)	₹27K	₹1L	₹2.3L	₹4L	₹5.7L	₹7.3L
Subscriptions (Sakhi Pro)	50	400	1.2K	2.5K	4K	5.5K
Sub rev/mo (₹)	₹10K	₹80K	₹2.9L	₹6.2L	₹10L	₹13.8L
TOTAL MRR (₹)	₹0.97L	₹3.5L	₹12.2L	₹24.2L	₹37.7L	₹53.1L
ARR Run-Rate (₹Cr)	₹0.12	₹0.42	₹1.46	₹2.90	₹4.52	₹6.37

KEY ASSUMPTIONS: Avg screening ticket ₹2,000 (mid of ₹1,500-3,000 range) | B2B ₹800/emp/yr avg (mid of ₹500-1,200) | Sakhi Pro ₹249/mo | Screening conversion 5% of MAU annually | D30 retention 20% | **\$1M ARR (₹8.5Cr) reached between M28-32 in base case.**

What Breaks + What We Do About It

Scenario	Change	Impact on M30 ARR	Countermeasure
BASE CASE	As modeled	₹4.52Cr (\$532K)	N/A
Retention drops 30%	D30: 20% → 14%	₹2.8Cr (\$329K)	Double down on community features + habit mechanics; increase personalization investment
CAC increases 50%	Avg ₹120 → ₹180	₹3.9Cr (\$459K)	Shift budget to organic + B2B; reduce paid to 15% of mix
Screening conv. halved	5% → 2.5% annual	₹2.9Cr (\$341K)	Increase subscription monetization; add lab partnership incentives (bundle discounts)
B2B deals 50% slower	28 → 14 clients by M36	₹3.7Cr (\$435K)	Hire dedicated B2B sales person M6; offer 60-day free pilots
WORST CASE (all)	All above combined	₹1.5Cr (\$176K)	Extend timeline to 48 months; seek bridge funding

WHAT BREAKS FIRST

- 1. **Retention is the #1 risk.** If D30 retention drops below 14%, the engagement loop breaks and screening conversion collapses. This is why we index on WAU, not installs.
- 2. **Trust breach is existential.** One data leak or misleading health claim kills the brand in women's health. Zero tolerance policy on data handling.
- 3. **Lab partner quality.** If screening experience is bad (late reports, rude phlebotomists), users blame Sakhi. Strict SLAs + user feedback loop.

Key Bets + What We Will NOT Do in Year 1

WE WILL (Year 1 Bets)

- ✓ Start urban, Tier 1-2 cities only (Bangalore, Mumbai, Pune, Delhi NCR)
- ✓ Lead with wellness, earn trust before screening asks (Day 21+ only)
- ✓ Build B2B corporate channel from Month 3 as primary revenue anchor
- ✓ Run 3 high-signal experiments in first 6 months to validate core assumptions
- ✓ Maintain transparent pricing — no hidden margins on lab services
- ✓ Invest in content-led organic (40% of marketing budget) for long-term CAC advantage
- ✓ Hire community manager before growth marketer (trust > scale)

WE WILL NOT (Year 1)

- ✗ Expand to rural/semi-urban (deferred to Year 2+ with partner-led model)
- ✗ Optimize ARPU aggressively — floor pricing, focus on volume and trust
- ✗ Sell or monetize user health data to any third party — ever
- ✗ Build our own diagnostic labs — partner-led model only
- ✗ Spend >30% of budget on paid acquisition — organic-first
- ✗ Add male users or general health features — women-only focus
- ✗ Launch a mobile app for rural — WhatsApp-first for Tier 3+ later

REFLECTIONS

Execution Risks, Team Fit, Would We Invest?

REAL-WORLD EXECUTION RISKS

1. Lab partner SLA failures (bad sample collection experience) can tank NPS overnight
2. Health misinformation liability — one wrong health tip goes viral = brand death
3. Regulatory ambiguity (DPDP Act 2023) on women's health data handling in India
4. User fatigue from wellness apps (competing with HealthifyMe, Flo, cult.fit for attention)
5. B2B sales cycle longer than modeled — enterprise procurement is slow and political

WHY THIS CAN SUCCEED

Massive unmet need (1.9% screening) + tailwind (18% CAGR femtech India) + no dominant player in wellness-to-screening bridge. The progressive commitment model is defensible because competitors either do wellness (HealthifyMe) OR diagnostics (Practo/Healthians) — nobody bridges both. First mover in the bridge = category creator.

WOULD WE INVEST OUR OWN MONEY?

Yes, with conditions: (1) Founder must have health-domain credibility or co-founder from diagnostics, (2) First 500 users must validate the wellness→screening bridge with >3% conversion, (3) One signed B2B pilot before seed round.

TEAM ROLES WE'D FIT

Growth Lead: Owns funnel metrics, experiment design, channel mix optimization, and North Star tracking

B2B Partnerships: Leads corporate outreach, pilot execution, HR relationship management, and case study development

Content & Community: Creates culturally resonant health content in Hindi/English; manages community challenges and trust-building

Product & Data: Builds risk profiling engine, personalization algorithms, health dashboards, and A/B testing infrastructure

Appendix

Interview Questions + 72-Hour Execution Plan

USER INTERVIEWS (8 Questions)

1. When did you last think about your health unprompted? What triggered it?
2. Have you ever been screened for breast/cervical cancer? If not, what stopped you?
3. What health apps do you currently use? What keeps you coming back?
4. If an app told you your health risk profile, would you act on it? Why/why not?
5. Who influences your health decisions most — family, doctor, friend, social media?
6. Would you share a health tip or challenge with your WhatsApp group? What kind?
7. What's the most you've paid for a health service in the last year? Was it worth it?
8. What would make you trust a NEW health platform enough to share personal health data?

DOCTOR / LAB PARTNER (4 Questions)

1. What % of your female patients come for preventive screening vs. symptomatic?
2. What's the biggest barrier you see to women getting screened regularly?
3. Would you partner with an app that sends you screened referrals? What commission works?
4. What screening packages do your women patients most commonly need/request?

HR / CORPORATE BUYERS (3 Questions)

1. Do you currently offer women-specific wellness benefits? What's the annual budget per employee?
2. What would make you pilot a new women's health platform — outcomes data, price, ease of setup?
3. How do you measure ROI on wellness programs — absenteeism, engagement surveys, retention?

72-HOUR MINIMUM VIABLE RESEARCH PLAN (10 Interviews)

Day 1: Post recruitment calls on LinkedIn (target: HR heads) + WhatsApp groups (target: women 25-45). Use Google Form screener. Recruit 15 candidates for 10 slots.

Day 2: Conduct 6-8 user interviews (20 min each, Zoom/phone). Record with consent. 1-2 doctor/lab calls via Practo or personal network.

Day 3: Conduct remaining 2-4 interviews including 1-2 HR calls. Synthesize findings into persona validation + assumption update matrix. Tag each finding as "confirms" or "challenges" hypothesis.